

PUBLIC HEALTH LEADERSHIP

COMPETENCY MAPPING & WORKFORCE TRAINING AGENDA

2024

ASPPH ASSOCIATION OF SCHOOLS & PROGRAMS OF PUBLIC HEALTH
THE VOICE OF ACADEMIC PUBLIC HEALTH

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ACKNOWLEDGEMENTS

The ASPPH Public Health Workforce Development Taskforce advises ASPPH on the development of national training agendas for public health leadership, data science, and applied practice for the Public Health Leadership and Education, Advancing Health Equity and Data Science initiative, which is supported through funding from the Centers for Disease Control and Prevention (CDC) through Cooperative Agreement NU36OE000014 awarded to ASPPH, under funding opportunity OE22-2201 "Strengthening the Population & Public Health Workforce Pipeline."

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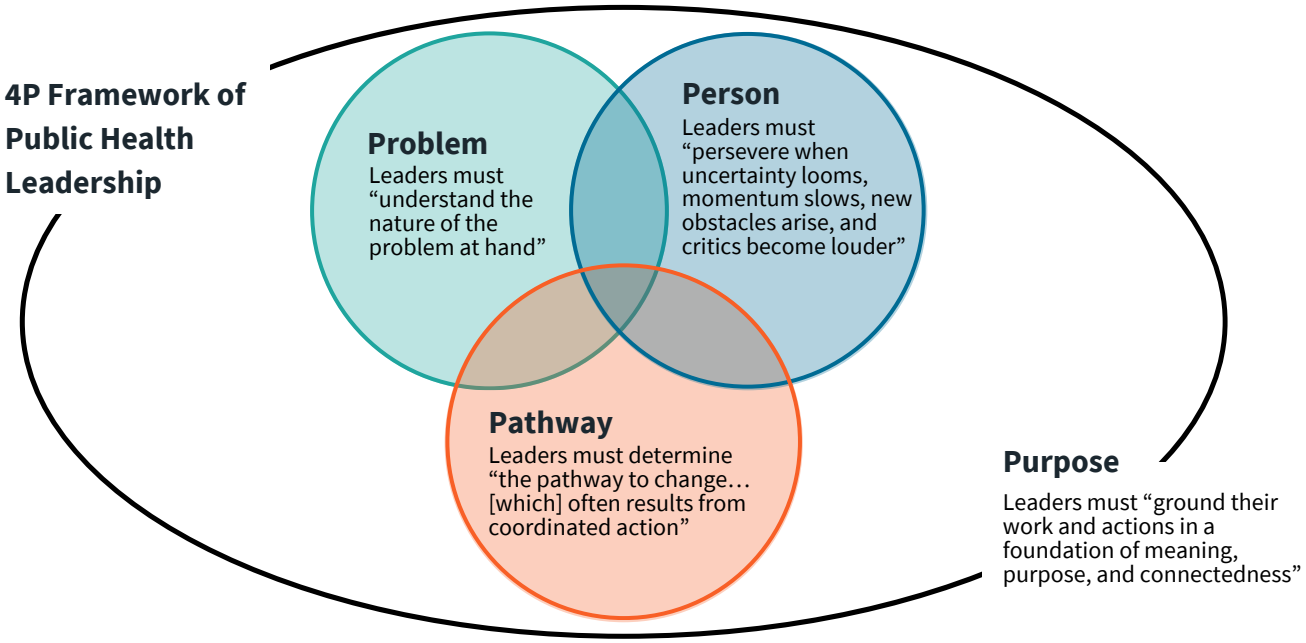
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EXECUTIVE SUMMARY

The COVID-19 pandemic laid bare the critical role of the governmental public health workforce and the challenges it faces in responding to public health emergencies, both in the United States and abroad. Prominent among these challenges are public health workforce training, development, and sustainability (Carlin et al., 2021, p. 92-93). Further compounding these challenges are politicization and harassment aimed at public health leaders. A longitudinal survey of public health workers involved in pandemic response from 2020 to 2021 found that one in three governmental public health leaders retired, resigned, or were dismissed from their positions leaving vacancies in critical positions of leadership (Horney et al., 2023, p. 1-2).

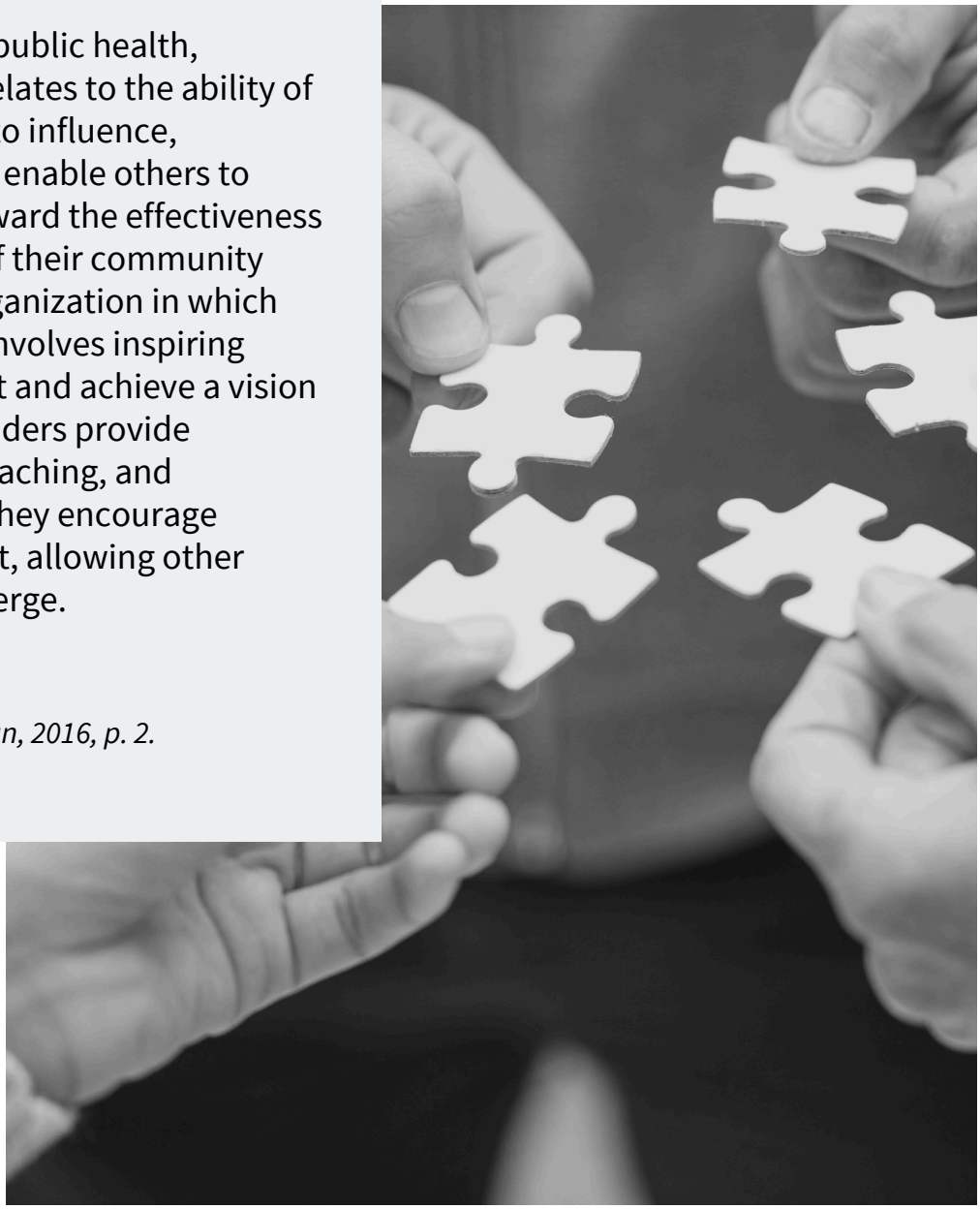
In 2022, the Association of Schools and Programs of Public Health (ASPPH) received funding from the U.S. Centers for Disease Control and Prevention (CDC) to engage academic public health and our practice partners to establish a national leadership training agenda for governmental public health. Bringing together an expert panel representing diverse perspectives, ASPPH developed the Public Health Leadership Competency Mapping and Training Agenda to establish a framework for the application of training that will increase the leadership competencies of the current and future public health workforce.

To achieve this goal, ASPPH studied published public health competencies, strategic frameworks, accreditation requirements, and job tasks and domains to identify and map leadership skills and competencies needed to address the changing public health landscape. After reviewing this mapping, the expert panel developed the 4P Framework of Public Health Leadership, modified from the 3P Framework for Social Innovators (Battilana et al., 2018), aligning skills and competencies into three overlapping areas with the foundation of purpose:



Using this modified framework and resulting skill and competency mapping, the ASPPH Public Health Leadership Expert Panel proposes in the following report a national training agenda for public health leaders both in positions of authority and positions of influence without direct authority. The national training agenda can be used by academic institutions offering public health degrees and certificates as well as public health employers, including state, tribal, local, and territorial public health departments.

> In the field of public health, [leadership] relates to the ability of an individual to influence, motivate, and enable others to contribute toward the effectiveness and success of their community and/or the organization in which they work. It involves inspiring people to craft and achieve a vision and goals. Leaders provide mentoring, coaching, and recognition. They encourage empowerment, allowing other leaders to emerge.
>
> *Robinson-Vollman, 2016, p. 2.*



PUBLIC HEALTH LEADERSHIP

INTRODUCTION



Public health is about adapting to change – there is no "status quo" (Rowitz, 2018). The health landscape of the world is constantly in flux, and the increasing threats to our global well-being have prompted the call for more leaders in the field of public health and have reinvigorated calls for leadership education and training within the field (Koh & Jacobson, 2009; Koh, 2009). The nature of the public health field makes it essential that leaders have skills in interdependence and handling uncertainty, as well as a willingness to face conflicts head-on. Public health problems pose specific challenges, creating a dire need for professionals trained in leadership who can guide the public through turbulent times. Effective leaders are crucial in solving unprecedented public health challenges and achieving progress in the field overall (Czabanowska & Babich, 2015).

The public health workforce is diverse, coming from various demographic, educational, and professional backgrounds (Coronado et al., 2019). Further, public health professionals span multiple occupations, including health educator, public health nurse, environmental scientist, biostatistician, epidemiologist, and more (Centers for Disease Control, 2022). Public health professionals must be comfortable – no matter their position or seniority – taking on leadership responsibilities. With increasing demands on public health professionals, combined with turnover and retirements in the workforce, the need for trained leaders in the field of public health is acute (Lachance & Oxendine, 2015). While the overarching theme of leadership is sweeping, incorporating specific skills and competencies into public health curricula will prepare students and practitioners to succeed in future public health leadership roles.

Despite multiple initiatives to build leadership skills in public health professionals, public health has failed to systematically ensure that leadership and management skills are essential components of public health training (Moodie, 2016). Those seeking specialized training in the discipline of public health leadership may turn to the lessons generated in other sectors, such as business or government. However, this approach overlooks the special nature of public health problems, as well as the unique opportunities that make public health leadership a source of inspiration (Koh & Jacobson, 2009). Public health problems are often enormous in scale, impact populations broadly, and regularly fall outside the control of a single authority.

Research shows that students planning to pursue a career in public health would benefit from leadership training, but reaching future professionals can be difficult – the public health workforce is broadly defined, with individuals coming from different disciplines (Hawley, 2021; Rowitz, 2018). Indeed, many people in the current public health workforce have little to no formal public health training – only around 14% of the workforce has a formal degree in public health (Rowitz, 2018; Hawley, 2021). In preparing students to become skilled and agile leaders for public health practice, leadership skills and competencies must be acquired and honed via education or training (Jadhav et al., 2017; Sell et al., 2022).

The American Public Health Association (APHA) states that public health leadership markedly functions to "convene people to solve complex problems" in the following domains: systems-based views, collaborative/team-based organizational cultures, diversity, community centrism and cultural humility, transdisciplinary approaches, and social movement building (APHA, 2022). "[Public health leadership] must also be bold, creative, and courageous in its commitment to promoting community-based public health" (APHA, 2022, p. 10).

There is a great need to develop leaders in public health at every level via interdisciplinary and interprofessional education and training (Czabanowska & Babich, 2015). Public health practitioners must be equipped with leadership skills to be effective in the complicated public health environment (Lachance & Oxendine, 2015). Leadership education and training should focus on the skills needed to effect change independent of distinct authority, and leadership development initiatives should be practice-based, process-based, interdisciplinary, adaptive, innovative, and should encourage authenticity (Lachance & Oxendine, 2015). Leadership skills and competencies must be incorporated into curriculum and training in public health and remain a focal point for professional development and continuing education among the governmental public health workforce. This will shape students and practitioners into leaders prepared to function in complex environments, effectively communicate with various stakeholders, lead diverse teams, and innovate in addressing critical public health issues.

Public health is currently at a crossroads where leadership skills at every level of the workforce will substantially enhance the progress of the profession. The increase of public health threats occurring globally has increased both the work and visibility of the public health workforce, making it essential that public health leaders have effective communication skills, with both the general public and the news media. The nurturing of the public health workforce, particularly with respect to the training of public health leaders, is one of the most important factors in strengthening the public health system in the U.S. and in countries around the world (Uno & Zakariasen, 2010). Due to anticipated

retirements from the field and the need for public health professionals to lead via interdisciplinary and interprofessional teams, schools and programs of public health should make leadership development a core aspect of curricula (Lachance & Oxendine, 2015).

Public health leadership, while often taught through theories of leadership and management, does not fully prepare students to implement strategies within practice (Public Health Accreditation Board, n.d.; Reid & Dold, 2017; Moodie, 2016). Theory alone will not prepare students and practitioners for leadership positions in the increasingly challenging public health landscape. However, competencies framed in theory will better equip those developing and delivering leadership curricula, trainings, and professional development.



BACKGROUND

To develop the forthcoming public health leadership competency mapping and training agenda, ASPPH conducted a review of current literature. Articles were identified from peer-reviewed journals and from grey literature, published in English, within the lens of academic and practice-based public health and health collaboration in the United States between the years of 1988 and 2023. The literature were then organized into three thematic areas:

1. Frameworks in public health leadership
2. Recommended knowledge, skills, and abilities of public health leaders
3. Recommended and applied modalities for teaching & training public health leadership

Much of the literature on the topic of public health leadership development credits the 1988 Institute of Medicine (IOM) report, The Future of Public Health, as a major catalyst for the examination of and efforts to improve public health leadership education. In the report, the IOM writes, “Today the need for leaders is too great to leave their emergence to chance.” (Institute of Medicine, 1988). This statement remains poignant.

A key theme in The Future of Public Health was education, including the need for improved preparation of leaders, more opportunities for continuing education, and great attention to the practice of public health by academia. CDC addressed these needs by funding schools and programs of public health to develop leadership training programs for public health officials, and to St. Louis University specifically to create the National Public Health Leadership Development Network (NLN) – a consortium of institutes providing a system for leadership development. In 2000, NLN made one of the earliest efforts to focus interest on public health leadership. The institute identified 79 leadership competencies through the use of sequential workgroup assignments to NLN members (Jadhav et al., 2017). By 2008, the NLN included more than two dozen state and regional leadership institutes that served almost all states in the U.S., a national institute, and several international institutes (Reid & Dold, 2017).

The curricula of the one- to two-year programs within the network were designed to enhance leadership competence in facilitating performance of the essential services of public health practice (Wright et al., 2000). In turn, enhanced leadership competence advances individual and organizational competence in improving and promoting the health of communities in the U.S. (Wright et al., 2000). NLN’s work group and advisory board projects have resulted in: development

of a conceptual model for design of leadership programs; technical assistance for development of program models; and development of the Leadership Competency Framework (Wright et al., 2000).

The Leadership Competency Framework was created as a baseline set of competencies over the 1995-2000 time period. The NLN members recognized the importance of a leadership competency set for "the acquisition and measurement of knowledge, skills, and competencies" (Reid & Dold, 2017). They agreed that identifying the development and competency needs of those who aspire to or currently hold public health leadership positions was a priority (Wright et al., 2000). Members of NLN noted the need for a competency framework specific to professional preparation of public health leaders. Initially, the NLN Competency Work Group – consisting of members from practice and academic sectors – reviewed literature on leadership development and existing competency frameworks, including:

- Public Health Faculty/Agency Forum's Public Health Competencies
- University of North Carolina School of Public Health's* Doctorate in Public Health Leadership Competencies
- Johns Hopkins University School of Hygiene and Public Health's* Community Based Public Health Competencies
- ASPPH's Maternal and Child Health Competencies
- CDC's Public Health Core Functions and Essential Services

**Institution names may have changed since the publication of competency frameworks.*

In reviewing these frameworks, the work group began identifying major areas of leadership practice and corresponding competencies. The 2005 version of the framework listed 80 competencies in 4 categories: core transformational competencies, political competencies, transorganizational competencies, and team-building competencies (Begun & Malcolm, 2014).

The resulting competency set was designed as a guide for curriculum content and module development, and the framework was extensively used by the NLN members in designing their respective curricula (Wright et al., 2000; Reid & Dold, 2017). The NLN model is summarized by four clusters of competencies, but the large number of competencies (80) makes the model perhaps more appropriate for larger-scale and longer educational processes (Begun & Malcolm, 2014).

Following the accreditation initiative from the Public Health Advisory Board (PHAB), NLN developed accreditation standards for public health agencies. In 2007, PHAB released its first set of official guidelines including an emphasis on the creation of a competent workforce. In 2013, PHAB released version 1.5 of their accreditation standards, which included an intentional emphasis on leadership development (Jadhav et al., 2017).

In 2001, the Public Health Foundation (PHF)'s Council on Linkages Between Academia and Public Health Practice (COL) issued a set of competencies for public health practitioners. The competencies were designed to be compatible with the 10 Essential Public Health Services listed in 1994 by the Core Public Health Functions Steering Committee. The competencies in this

framework were defined by three tiers: entry, program management/supervisory, and executive/senior management. This framework was updated in 2010, when the COL developed eight competency domains: analytical/assessment, policy development/program planning, communication, cultural competency, community dimensions of practice, public health sciences, financial planning and management, and leadership/systems thinking (Begun & Malcolm, 2014).

The Turning Point initiative, funded by the Robert Wood Johnson Foundation and operated between 1997 and 2006, was comprised of a network of 23 state partners, CDC, PHF, APHA, Association of State and Territorial Health Officials (ASTHO), and the National Association of Local Boards of Health (NALBOH). The leadership perspective selected was given the label Collaborative Leadership, which then developed into six Collaborative Leadership learning modules: assessing the environment; creating clarity (visioning and mobilizing); building trust; sharing power and influence; developing people; and self-reflection. The Turning Point framework emphasized a sharing of power, and the framework's emphasis on servant leadership and collaboration is notable (Begun & Malcolm, 2014).

In their 2014 book Leading Public Health: A Competency Framework, Begun and Malcolm outline their own competency framework for public health leadership. They first describe the circumstances specific for public health leadership, and further say that these circumstances suggest certain leadership approaches that are especially relevant for public health leaders:

SERVICE LEADERSHIP

Leadership activities should flow from a desire to serve; leadership activities are necessary for programs and organizations to succeed; servant leaders step into their role to enable their organizations to succeed; motivation comes from the goals of the program, organization, or community, not from their own goals.

ADAPTIVE LEADERSHIP

The practice of mobilizing people to tackle difficult challenges and thrive while doing so; enacting change that enables the capacity to thrive; builds upon past interventions rather than jettisoning them; relies on diversity and values diverse viewpoints (Heifetz et al., 2009). Adaptive challenges might take time and often require cultural shifts within organizations (WGU, 2021). This approach helps organizations and leaders adapt and thrive in challenging situations, and prepares them to take on the process of change. This approach involves innovating, diagnosing, and interrupting as a way to create change that coincides with the goals of an organizations (WGU, 2021).

INTEGRATIVE LEADERSHIP

Bringing diverse organizations and groups together – typically across various sectors – to remedy complex problems and achieve a common good (Crosby & Bryson, 2010).

COMPLEXITY LEADERSHIP

The process of fostering conditions where the new direction and behaviors of an organization can emerge via regular, dynamic interaction rather than trying to control or direct what happens within the organization. Complexity leadership involves influencing organizational behavior through the management of interactions and networks (Brown, 2011).

These leadership approaches provide guidance and philosophy for individuals, but in order to be translated into action, leadership approaches must be communicated in the form of competencies or skills. Begun and Malcolm's competency framework involves five competency sets for a strong foundation for public health leadership:

1

Invigorate bold(er) pursuit of population health.

Emphasizes the need to create a strong foundation for public health programs by facing realities, making decisions among alternative paths, and bringing others in on the journey. Developing targeted and realistic goals and creating a strong base of resources are other needed competencies to create a solid foundation for action (Begun & Malcolm, 2014).

2

Engage diverse others in public health initiatives.

Engagement of others in public health work, as public health issues are typically complex and it is rare that any one program or group can solve them independent of others. Engaging others requires a broad search for collaborators, understanding their views and circumstances, outlining potential responses, and identifying which partners are best for a particular activity (Begun & Malcolm, 2014).

3

Effectively wield power to increase the influence and impact of public health.

Reinforces the need for public health to proactively advocate for its share of influence and resources. Wielding power can be both formal and informal authority, analyzing solutions in terms of building coalitions, dealing with opponents, and being agile strategically (Begun & Malcolm, 2014).

4

Prepare for surprise in public health work.

In public health, surprises and emergencies will occur, and planning for them in advance is the first step in adequate preparation. When surprises do happen, the communication skills of public health leaders may be tested. Practicing plans under numerous conditions, executing plans in emergency situations, and conducting postmortem reviews are the main competencies of preparing for surprising and unexpected situations (Begun & Malcolm, 2014).

5

Drive for execution and continuous improvement in public health programs and organizations.

Implementation for public health programs is difficult and necessary work. To mobilize others to take on public health challenges effectively requires concentration on the tools of execution and constant improvement. This might include holding others accountable, establishing and using metrics and targets, being responsible stewards of funds, conducting quality improvement, and encouraging innovation (Begun & Malcolm, 2014).

In 2015, Grimm et al. conducted a qualitative analysis to determine domains and skills necessary to effective leadership in the field of public health. Their analysis resulted in the review of around 161 skills, definitions, traits, and competencies from Transformational Leadership[1], Servant Leadership,[2] Emotional Intelligence,[3] Collaborative Leadership[4], and Appreciative Leadership[5] models, as well as the NLN competency framework (Grimm et al., 2015). The below domains and skills needed for leadership were the result of this analysis.

1. **Community/Organizational Responsiveness:** The ability to be keenly aware of the community and system around you. Skills: to serve the community; to connect individual work/actions and the impact on the larger organization, community, and/or system; and to make decisions for the improvement of the larger organization, community, or system.

2. **Ability to Inspire:** The ability to relate to others in a way that brings out the best in them. Skills: collaborate and promote team work; build warm caring relationships; and to mentor/develop others.

3. **Results Focused:** The ability to focus on outcomes and achievement of results. Skills: communicate purpose and vision leading to results; be decisive in decision making; offer clear direction; and think strategically and communicate strategy to others.

4. **Social Intellect:** The ability to listen and engage with others. Skills: effectively engage in conflict and controversy, manage personal feelings, listen to others' concerns and deal with their feelings, and share power and influence with others.

5. **Authenticity:** The ability to be true to one's own personality, spirit, character, and ethics. Skills: lead with honesty, integrity, and trustworthiness; willingness to stand for individual beliefs; and deal with and discuss difficult problems and situations.

6. **Composure and Balance:** The ability to use a level of selflessness and remain composed and balanced while in the center of high-pressure situations. Skills: balance stresses of work and life healthily; be humble; remain composed and calm in the heart of conflict and change; and make time for continued learning and development
(Grimm et al., 2015).

Below are additional public health leadership training initiatives utilizing public health leadership competency frameworks. A thorough evaluation of these initiatives could provide additional insights as to which methods are most successful in public health leadership training programs.

National Public Health Leadership Institutes

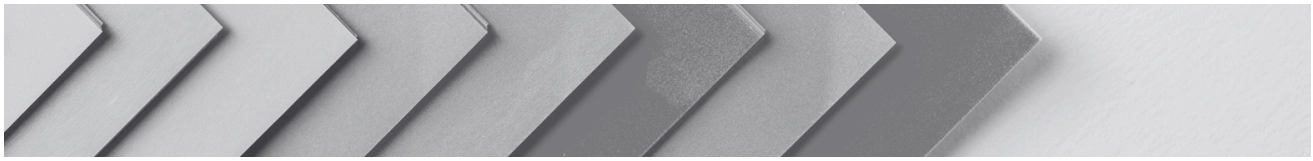
- Sponsored by the CDC, the National Public Health Leadership Institute (PHLI) was created in 1990. PHLI's aim was to nurture collaborative leaders and to strengthen networks of leaders who share knowledge and address public health problems (Umble et al., 2005).
- Evaluation results of PHLI show that completing the institute's training increased collaborative leadership and enhanced knowledge sharing and problem-solving networks. This, in turn, strengthened interorganizational relationships, coalitions, services, programs, and policies (Umble et al., 2005). Evaluation also supports the idea that networks and collaborative leaders can improve services and programs (Umble et al., 2005).

Region IV Public Health Leadership Institute

- Supported by the Health Resources and Services Administration (HRSA) and headquartered at the Rollins School of Public Health at Emory University, the Region IV PHLI provides training for emerging leaders who work in state, local, or tribal public health departments/tribal health organizations or as primary care providers in the eight states that comprise the HHS Region IV (Alabama, Florida, Georgia, Kentucky, Mississippi, North Carolina, South Carolina, Tennessee).
- Region IV PHLI's purpose is to empower these public health professionals with the adaptive and strategic leadership skills needed to address the social, community-based, and economic determinants of health.
- Over eight months, fellows participate in an in-person retreat and seven virtual leadership development sessions. Fellows complete intersession work independently between the group virtual sessions that focuses on peer learning and applying adaptive leadership skills in local contexts (JW Fanning Institute for Leadership Development, n.d.).

Region VI Public Health Leadership Institute

- The Region VI South Central Public Health Training Center launched the Region VI PHLI in the Fall of 2023. Headquartered at the Tulane School of Public Health and Tropical Medicine, its purpose is to develop the next wave of leaders in public health by providing an opportunity for strategic professional growth for those interested in refining their leadership skills and advancing their career.
- Region VI PHLI is designed as a learning community that explores timely leadership topics and builds core skills. The sessions use interactive discussions with leaders in the field and are reinforced via practice activities. The curriculum was created using focus groups and key informant interviews with professionals across the five states (Arkansas, Louisiana, New Mexico, Oklahoma, Texas) of Region VI to identify issues and the leadership skills currently needed in practice (Region 6 Public Health Leadership Institute , n.d.).



North Carolina Public Health Leadership Institute

- Housed within the University of North Carolina Gillings School of Global Public Health, the North Carolina Public Health Leadership Institute is a nine-month, intensive leadership development program for state and local public health practitioners in North Carolina.
- Participants in the program learn the essential skills of adaptive leadership, learn from experts, build connections with peers from across the state, and apply leadership skills to real-world challenges (UNC Gillings School of Public Health, n.d.).

Northwest Public Health & Primary Care Leadership Institute

- Developed by the Northwest Center for Public Health Practice and the Northwest Regional Primary Care Association, the Leadership Institute brings together future leaders in public health and primary care to develop equitable and effective approaches to population health issues.
- Centering health equity, the Institute creates cross-sector communities of practice and develops participant competencies in: population health and health equity, systems thinking, leadership and change management, and healthcare transformation and system integration.
- The Institute utilizes problem-based learning, an instructional approach that mimics real-world problems to drive the learning process (Northwest Center for Public Health Practice, 2019).

Center for Leadership in Public Health Practice

- The Center for Leadership in Public Health Practice began at the University of South Florida College of Public Health in 1999. The Center was established to give organizational structure to a number of activities within the College, including training for the Florida Department of Health workforce.
- The Center's mission is to develop leaders who advance the public's health by creating the conditions for the regenerative ecosystems necessary to ensure thriving for all (USF Health College of Public Health, n.d.).

Center for Health Leadership

- The Center for Health Leadership (CHL) at the UC Berkeley School of Public Health prepares students to become effective professionals and leaders by developing the skills that will allow them to lead from any role within an organization. CHL has used a theme of "leading from where you are" to allow students to engage with leadership throughout their career, irrespective of their positional authority, to impart change and contribute to the goals of a community, project, or organization (Lachance & Oxendine, 2015).
- CHL's framework is anchored by a leadership competency set that was developed on the basis of several existing competency frameworks and is informed by the 10 guiding principles that provide a framework for their approach to leadership development activities (Lachance & Oxendine, 2015).



PUBLIC HEALTH LEADERSHIP COMPETENCY MAPPING

4P FRAMEWORK OF PUBLIC HEALTH LEADERSHIP

As evidenced by existing public health leadership frameworks, public health as a field has made progress in identifying important themes for education and leadership development. While there is progress in public health leadership education and training, further advances can and should be made to address public health infrastructure challenges. Public health leaders must know how to navigate complexity and have the ability to understand and accept failure, while acknowledging that those in positions of authority cannot be the only problem solvers within a team or organization.

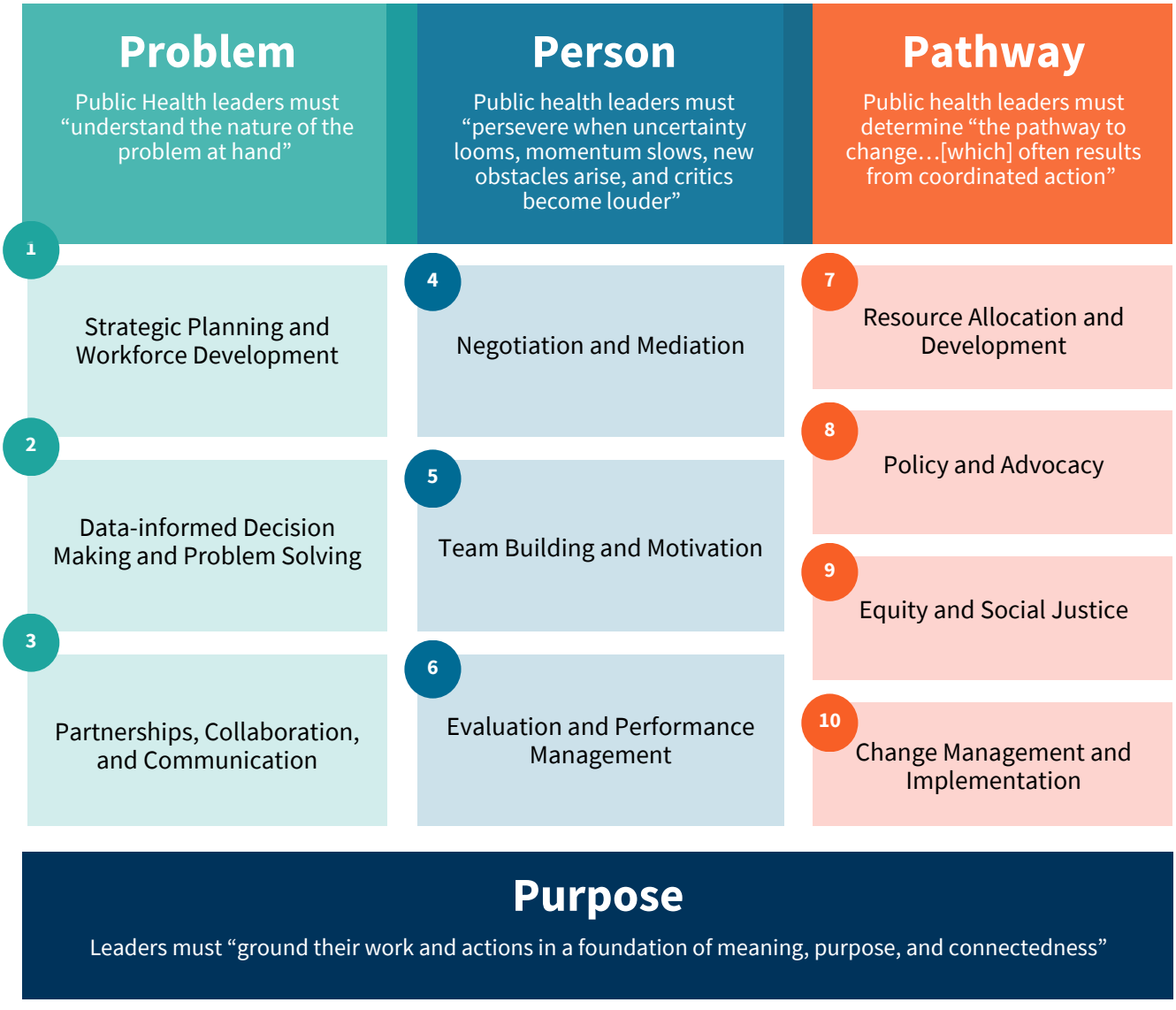
The Problem, Person, and Pathway Framework for Social Innovators (3P Framework) is a proposed theoretical approach to innovation that can guide public health leaders to more creative solutions to critical public health issues as it accounts for ways in which different contexts call for different responses as well as the politics of change (Battilana et al., 2018). The 3P Framework was initially designed to better equip social innovators in tackling significant societal issues. As such, it translates well to public health leaders addressing significant public health issues and was adapted by the ASPPH Public Health Leadership Expert Panel to also include Purpose, thus creating the 4P Framework for Public Health Leadership.

When applied to public health, the 4P Framework emphasizes the importance of thoroughly understanding the nature of the **problem** at hand. Public health leaders are encouraged to immerse themselves in the context of the problem, considering various contributing factors and community and partner perspectives. The 4P Framework explores the motivations and sources of power behind the **person** driving change. Understanding one's capacity for influencing others is crucial for effective public health leadership, underscoring the importance of building coalitions and aligning motivations to mobilize collective action towards change. The framework explores different **pathways**, including both individual action and collaboration within existing structures and communities, to maximize impact and sustainability.

The 4P Framework for Public Health Leadership requires an iterative process, where public health leaders continuously revisit their assumptions and strategies in response to evolving circumstances and requires flexibility, reflection, and adaptation in navigating the complexities of addressing public health challenges. Adapting this 3P Framework to an expanded 4P Framework of Public Health Leadership provides leaders with direction for investigation, problem solving, and innovation, wherein the ecosystem becomes the **purpose** of the vocation of public health. Exploration of strengths, weaknesses, values, purpose, and motivations, establishes a foundation

to develop personal and systemic mastery and connection to public health work (Koh et al., 2023). This mastery elucidates purpose in the vocation and enables public health leaders to guide teams, organizations, and systems, even during volatility and uncertainty.

The ASPPH Public Health Leadership Expert Panel proposes that the 4P Framework for Public Health Leadership serves as the scaffolding for the public health leadership competency mapping and training agenda detailed in this report. Working with the panel, researchers referenced published public health competencies, strategic frameworks, accreditation requirements, and job tasks and domains to identify and map leadership competencies and skills needed to address the changing public health landscape. A keyword search was performed among all documents for the term "leadership", which was followed by a complete scan to identify leadership-adjacent terms and concepts. For each identified leadership competency, researchers analyzed additional competency information, including description, key knowledge and/or skills. The resulting leadership competencies were analyzed for key themes. Ten key themes or areas were identified and segmented into Problem, Person, or Pathway, all existing within the ecosystem of Purpose in the 4P Framework of Public Health Leadership.



COMPETENCIES WERE EXTRACTED FROM THE FOLLOWING:

Council on Education for Public Health 2021 Accreditation Criteria
The Council on Education for Public Health (CEPH)'s 2021 Accreditation Criteria includes the criteria necessary for a school or program of public health to become an accredited institution and the Foundational Competencies of Public Health Bachelor's, MPH, and DrPH degrees.

Core Competencies for Public Health Professionals
The Core Competencies are a consensus set of skills and knowledge for the practice of public health, as defined by the 10 Essential Public Health Services. These competencies, created by the PHF Council on Linkages Between Academia and Public Health Practice (COL), reflect foundational and cross-cutting knowledge and skills for all professionals engaging in public health education, research, and practice. The competencies are arranged into eight domains, which represent skill areas within public health, and three tiers, which illustrate different types of responsibilities within public health organizations.

National Board of Public Health Examiners 2016 Public Health Job Task Analysis
This job analysis study was initiated by the National Board of Public Health Examiners (NBPHE) in 2014. Its purpose was to provide a detailed analysis of the job-related competencies required of those working in the field of public health.

10 Essential Public Health Services
The 10 Essential Public Health Services provide a framework for public health to protect and promote the health of all people in all communities. The Essential Public Health Services actively promote policies, systems, and overall community conditions that enable optimal health for all and seek to remove systemic and structural barriers that have resulted in health inequities.

Public Health Accreditation Board Standards and Measures
The Public Health Accreditation Board (PHAB)'s Standards and Measures for Initial Accreditation serves as the official set of standards, measures, needed documentation, and guidance blueprint for PHAB national public health department initial accreditation. The "Standards" referenced in the document refer to the entire document including the introductory material, domains, standards, measures, required documentation, and guidance. They also provide requirements and guidance for public health departments in preparing for their initial accreditation. It also serves anyone offering consultation or technical assistance to health departments preparing for accreditation, and guides PHAB's Board of Directors and staff as they administer the accreditation program.

HOW TO READ THE COMPETENCY MAPPINGS

The three forthcoming tables detail the identified leadership competencies, segmented vertically into the key themes or areas for public health leadership training. Competencies that fall under multiple key themes or areas are merged horizontally across the table. If the competency belongs within multiple P's, this is indicated by ***italicized*** text. The superscript appearing at the end of each competency indicates the source.



PROBLEM: LEADERS MUST UNDERSTAND THE PROBLEM

1	2	3
Strategic Planning and Workforce Development	**Data-Informed Decision Making and Problem Solving**	**Partnerships, Collaboration, and Communication**
Guide organizational decision-making and planning based on internal and external assessments[3]		
Create a strategic plan[1]	Create opportunities for creativity and innovating[2]	Create opportunities to achieve cross-sector alignment[2]
Develop a mission, goals, and shared vision for an organization or the community in conjunction with key stakeholders[3]	Facilitate shared decision making through negotiation and consensus-building methods[1]	Communicate an organization's mission, goals, values, and shared vision to stakeholders[3]
	Communicate public health science to diverse stakeholders, including individuals at all levels of health literacy, for purpose of influencing behavior and policies[1]	
Develop a workforce development plan that assesses workforce capacity and includes strategies for improvement[5]		
Provide professional and career development opportunities for all staff[5]		
Build and support a diverse and skilled public health workforce[4]		
Develop capacity-building strategies at the individual, organizational, or community level[3]	*Develop strategies to motivate others for collaborative problem solving, decision-making, and evaluation*[3]	Develop strategies for collaboration and partnership among diverse organizations to achieve common public health goals[3]
		Propose strategies for health improvement and elimination of health inequities by organizing stakeholders, including researchers, practitioners, community leaders, and other partners[1]
Build and maintain a strong organizational infrastructure for public health[4]		Implement strategies for collaboration and partnership among diverse organizations to achieve common public health goals[3]
		Integrate knowledge, approaches, methods, values and potential contributions from multiple professions, sectors, and systems in addressing public health problems[1]

PERSON: LEADERS MUST UNDERSTAND THEIR ROLE AND INFLUENCE

4	5	6
Negotiation and Mediation	**Team Building and Motivation**	**Evaluation and Performance Measurement**
Facilitate shared decision making through negotiation and consensus-building methods[1]	*Develop strategies to motivate others for collaborative problem solving, decision-making, and evaluation*[3]	
	Create teams for implementing health initiatives[3]	Establish and model standards of performance and accountability[3]
		Implement strategies to support and improve team performance[3]
		Assess one's own strengths and weaknesses in leadership capacities, including cultural proficiency[1]
		Prepare professional development plans for self or others[3]
Apply negotiation and mediation skills to address organizational or community challenges[1]		

PATHWAY: LEADERS MUST DETERMINE THE PATHWAY TO CHANGE

7	8	9	10
Resource Allocation and Development	**Policy and Advocacy**	**Equity and Social Justice**	**Change Management and Implementation**
Propose human, fiscal, and other resources to achieve a strategic goal[1]		*Build and support a diverse and skilled public health workforce*[4]	Create organizational change strategies[1]
Cultivate new resources and revenue streams to achieve a strategic goal[1]			Manage organizational change[2]
	Engage politicians, policymakers, and the public to support public health infrastructure[2]	Propose strategies to promote inclusion and equity within public health programs, policies, and systems[1]	Integrate knowledge, approaches, methods, values and potential contributions from multiple professions, sectors, and systems in addressing public health problems.
		Propose strategies for health improvement and elimination of health inequities by organizing stakeholders, including researchers, practitioners, community leaders, and other partners[1]	
	Analyze political, social, and economic policies that affect health systems at the local, national, or global levels[3]	Design public health interventions that incorporate such factors as gender, race, poverty, history, migration, or culture within public health systems[3]	Address facilitators and barriers impacting delivery of the 10 Essential Public Health Services[2]
		Apply social justice and human rights principles with addressing community needs[3]	Implement a vision for a healthy community[2]
		Assess one's own strengths and weaknesses in leadership capacities, including cultural proficiency[1]	

PURPOSE: LEADERS MUST BE GROUNDED IN A FOUNDATION OF MEANING, PURPOSE, AND CONNECTEDNESS

While understanding one's purpose and capacity for influence are critical, the 4P Framework for Public Health Leadership suggests that leaders must be grounded in a foundation of meaning, purpose, and connectedness. These foundational elements are often connected to one's "spirituality", which need not be religious, rather leading with a sense of deep meaning, purpose and, a "connection with a cause or some significant entity that is greater than themselves" (Koh et al., 2023, p. 2). Spiritual leadership, an offshoot of value-based leadership, is defined as "the values, attitudes, and behaviors necessary to intrinsically motivate self and others so that they have a sense of spiritual survival through calling and membership" (Koh et al., 2023).

Pursuit of connection with a cause that is greater than oneself is in fact a deeply personal journey. Further, meaning, purpose, and connectedness are not necessarily teachable skills, rather behavioral competencies linked to attitudes and beliefs. As such the ASPPH Public Health Leadership Expert Panel did not identify any previously existing competencies and skills that map to Purpose. However, the following competencies described in the literature for business leadership are modified from spiritual leadership competencies and proposed for public health leaders (Koh et al., 2023):

- Initiate regular contemplative practices that encourage mindfulness, awareness, and attention for self and staff, such as crafting authentic personal narratives about why one is motivated to serve as a public health leader.
- Take action to minimize work-related stress, optimize productivity, and foster overall satisfaction and fulfillment for self and staff, including sharing practices for healing.
- Establish and advance organizational norms, practices, and behaviors that prioritize integrity, transparency, and accountability.
- Foster an inclusive environment where all individuals feel valued, supported, respected, and connected to the team and organization.



NATIONAL PUBLIC HEALTH LEADERSHIP TRAINING AGENDA

The competency mapping informed the development of the National Public Health Leadership Training Agenda for the governmental public health workforce, addressing emerging needs and trends related to public health workforce leadership training and curricula. In mapping the competencies, the ASPPH Public Health Leadership Expert Panel identified areas for training opportunities related to the key themes and competencies, calling for the identification of gaps in available trainings and the development of new training opportunities to address such needs.

Academic and professional development trainings should address multi-tiered workforce, community, and organizational needs or gaps with trainings grounded in case-based, systems thinking strategies.



Public Health Leadership Training

PROBLEM

Strategic Planning and Workforce Development

- ☐ Strategic planning, including development and monitoring of mission, goals, and shared vision with key partners and communities served
- ☐ Organizational management, including capacity-building strategies and workforce development planning

Data-Informed Decision Making and Problem Solving

- ☐ Assessment-driven organizational decision-making and planning
- ☐ Negotiation and consensus building for shared decision-making
- ☐ Ethical data practices

Partnerships, Collaboration, and Communication

- ☐ Equity-driven coalition building and community organizing with emphasis on cross-sector alignment
- ☐ Communication strategies, including crisis communication, persuasive communication, and public speaking

PERSON

Negotiation and Mediation

- ☐ Negotiation and mediation skills, including skills in facilitative leadership and psychological safety

Team Building and Motivation

- ☐ Effectively developing diverse teams through structured mentorship and coaching
- ☐ Human resources best practices and legal guidance
- ☐ Emotional intelligence for performance review and feedback techniques

Evaluation and Performance Measurement

- ☐ Self-assessing strengths and weaknesses as well as modeling of standards of performance and accountability
- ☐ Developing and implementing performance management systems to support and improve individual and team performance

Public Health Leadership Training

PATHWAY

Resource Allocation and Development

- ☐ Developing and managing mission-aligned budgets to monitor spending, control costs, and apply internal controls
- ☐ Identifying and securing innovative funding streams as well as building and maintaining positive relationships with funders
- ☐ Federal grant writing and compliance

Policy and Advocacy

- ☐ Understanding policymaking processes, policy agendas, and advocacy strategies
- ☐ Collective impact, coalition building, and public health messaging

Equity and Social Justice

- ☐ Cultural competency through an emancipatory approach, respecting diversity and individual experiences within groups (Wesp et al., 2018).
- ☐ Conducting cultural competency audits and developing and implementing equity and social justice roadmaps
- ☐ Cultivating a culture of equity and social justice through organizational policies, Health in All Policies (HiAP) framework, and social determinants of health

Change Management and Implementation

- ☐ Addressing facilitators and barriers impacting delivery of the 10 Essential Public Health Services
- ☐ Developing change management plans and strategies to engage employees
- ☐ Crisis management

PURPOSE

Meaning, Purpose, and Connectedness

- ☐ Personal and professional mindfulness for leaders
- ☐ Minimizing work-related stress for self and staff
- ☐ Establishing and advancing organizational norms, practices, and behaviors that prioritize integrity, transparency, and accountability



NOTES AND REFERENCES

NOTES

[1] "Emphasis on leaders who create change in deep structures, major processes, or overall culture. Leader's mechanisms may be a compelling vision, brilliant technical insight, and/or charismatic quality" (Grimm et al., 2015).

[2] "Emphasis on ethical responsibilities to followers, stakeholders, and society. Business theorists tend to emphasize service to followers, but political theorists emphasize citizens; public administration analysts tend to emphasize legal compliance and/or citizens" (Grimm et al., 2015).

[3] "The ability to manage ourselves and our relationships effectively consists of four fundamental capabilities: self-awareness, self-management, social awareness, and social skill. Each capability, in turn, is composed of specific sets of competencies" (Grimm et al., 2015).

[4] "The development process for collaborative leadership should consider three levels of focus to ensure effective working relationships within an ever-expanding sphere of influence
- Level 1: The individual and her or his leadership capacities
- Level 2: Individuals working within an organizational context
- Level 3: Community members working across boundaries to stimulate change and solve problems" (Grimm et al., 2015).

[5] "The relational capacity to mobilize creative potential and turn it into positive power—to set in motion positive ripples of confidence, energy, enthusiasm, and performance—to make a positive difference in the world" (Whitney, Trosten-Bloom, & Rader, 2010).

REFERENCES

American Public Health Association. (2022, November 8). *Reimagining Public Health Leadership for Health Equity: Moving Toward Collective and Community-Centered Applied Practice*. www.apha.org. https://www.apha.org/Policies-and-Advocacy/Public-Health-Policy-Statements/Policy-Database/2023/01/18/Public-Health-Leadership

Battilana, J., Butler, B., Kimsey, M., Mair, J., Marquis, C., & Seelos, C. (2018, May 25). *Problem, person, and pathway: A framework for social innovators*. Social Innovation + Change Initiative. https://sici.hks.harvard.edu/3p-framework-for-social-innovators/

Begun, J. W., & Malcolm, J. (2014). *Leading public health : a competency framework*. Springer Publishing Company.

Brown, B. (2011, October). *Learner Paper: Complexity Leadership - Integral Leadership Review*. Integral Leadership Review. https://integralleadershipreview.com/3962-learner-paper-complexity-leadership/

Carlin, M., Ensign, K., Person, C. J., Kittle, A., & Meadows, K. (2021). State of the Public Health Workforce: Trends and Challenges Leading Up to the COVID-19 Pandemic. *Journal of Public Health Management and Practice*, *27*(1), 92–93. https://doi.org/10.1097/phh.0000000000001294

Centers for Disease Control. (2022, October 24). *Public Health Career Spotlights | CDC NERD Academy Curriculum (for grades 6-12) | Science Ambassador Fellowship | CDC*. Www.cdc.gov. https://www.cdc.gov/scienceambassador/nerdacademy/careers.html

Coronado, F., Beck, A. J., Shah, G., Young, J. L., Sellers, K., & Leider, J. P. (2019). Understanding the Dynamics of Diversity in the Public Health Workforce. *Journal of Public Health Management and Practice*, *26*(4), 389–392. https://doi.org/10.1097/phh.0000000000001075

Council on Linkages Between Academia and Public Health Practice. (2021). *Core Competencies for Public Health Professionals Available from: phf.org/corecompetencies The Council on Linkages Between Academia and Public Health Practice.* https://www.phf.org/resourcestools/Documents/Core_Competencies_for_Public_Health_Professionals_2021October.pdf

Crosby, B. C., & Bryson, J. M. (2010). Integrative leadership and the creation and maintenance of cross-sector collaborations. *The Leadership Quarterly,* 21(2), 211–230. https://doi.org/10.1016/j.leaqua.2010.01.003

Czabanowska, K., & Babich, S. (2015). 8.D. Round table: “Whiter shades of pale” - public health leadership: policy, research, education and practice. *European Journal of Public Health, 25*(suppl_3). https://doi.org/10.1093/eurpub/ckv174.017

Grimm, B. L., Watanabe-Galloway, S., Britigan, D. H., & Schumaker, A. M. (2015). A Qualitative Analysis to Determine the Domains and Skills Necessary to Lead in Public Health. *Journal of Leadership Studies,* 8(4), 19–26. https://doi.org/10.1002/jls.21342

Hawley, S. R. (2021). Using adaptive leadership principles to support Public Health 3.0 in multidisciplinary undergraduate education. *Leadership in Health Services,* ahead-of-print(ahead-of-print). https://doi.org/10.1108/lhs-07-2020-0051

Heifetz, R. A., Grashow, A., & Linsky, M. (2009). The Practice of Adaptive Leadership: *Tools and Tactics for Changing Your Organization and The World.* Harvard Business Press.

Horney, J. A., Harjivan, A., Stone, K. W., Jagger, M. A., & Kintziger, K. W. (2023). Threats to public health workers. *Public Health in Practice, 6*, 100435. https://doi.org/10.1016/j.puhip.2023.100435

Institute of Medicine. (1988). *The Future of Public Health.* National Academies Press. https://doi.org/10.17226/1091

Jadhav, E. D., Holsinger, J. W., Anderson, B. W., & Homant, N. (2017). Leadership for Public Health 3.0: A Preliminary Assessment of Competencies for Local Health Department Leaders. *Frontiers in Public Health,* 5. https://doi.org/10.3389/fpubh.2017.00272

JW Fanning Institute For Leadership Development. (n.d.). *Public Health Leadership Institute Addressing community health issues through professional leadership development.* J.W. Fanning Institute for Leadership Development. Retrieved July 25, 2024, from https://www.fanning.uga.edu/programs/public-health-leadership-institute/

Koh, H. K. (2009). Leadership in public health. *Journal of Cancer Education,* 24(S2), S11–S18. https://doi.org/10.1007/bf03182303

Koh, H. K., & Jacobson, M. (2009). Fostering public health leadership. *Journal of Public Health,* 31(2), 199–201. https://doi.org/10.1093/pubmed/fdp032

Koh, H. K., Tso, C. C., Cyra Perry Dougherty, Lazowy, E. E., Heberlein, C. P., & Phelps, F. A. (2023). Exploring the spiritual foundations of public health leadership. *Frontiers in Public Health,* 11. https://doi.org/10.3389/fpubh.2023.1210160

Lachance, J. A., & Oxendine, J. S. (2015). Redefining Leadership Education in Graduate Public Health Programs: Prioritization, Focus, and Guiding Principles. *American Journal of Public Health*, *105*(Suppl 1), S60–S64. https://doi.org/10.2105/AJPH.2014.302463

Moodie, R. (2016). Learning about self: leadership skills for public health. *Journal of Public Health Research*, *5*(1). https://doi.org/10.4081/jphr.2016.679

Northwest Center for Public Health Practice. (2019, September 27). *Northwest Public Health & Primary Care Leadership Institute*. Northwest Center for Public Health Practice. https://www.nwcphp.org/training/northwest-public-health-leadership-institute

Public Health Accreditation Board. (n.d.). *Home*. Public Health Accreditation Board. https://phaboard.org

Region 6 Public Health Leadership Institute . (n.d.). *Region 6 Public Health Leadership Institute | Tulane PACE Learning Management System*. Pace.tulane.edu. https://pace.tulane.edu/leadership-institute/group/region-6-public-health-leadership-institute

Reid, W. M., & Dold, C. J. (2017). Leadership Training and the Problems of Competency Development. *Journal of Public Health Management and Practice*, *23*(1), 73–80. https://doi.org/10.1097/phh.0000000000000456

Robinson-Vollman, A. (2016, September 14). *Leadership in public health | Canadian Public Health Association*. Www.cpha.ca. https://www.cpha.ca/leadership-public-health

Rowitz, L. (2018). *Essentials of leadership in public health*. Jones & Bartlett Learning.

Sell, K., Hommes, F., Fischer, F., & Arnold, L. (2022). Multi-, Inter-, and Transdisciplinarity within the Public Health Workforce: A Scoping Review to Assess Definitions and Applications of Concepts. *International Journal of Environmental Research and Public Health*, *19*(17), 10902. https://doi.org/10.3390/ijerph191710902

Umble, K., Steffen, D., Porter, J., Miller, D., Hummer-McLaughlin, K., Lowman, A., & Zelt, S. (2005). The National Public Health Leadership Institute: Evaluation of a Team-Based Approach to Developing Collaborative Public Health Leaders. *American Journal of Public Health*, *95*(4), 641–644. https://doi.org/10.2105/ajph.2004.047993

UNC Gillings School of Public Health. (n.d.). *North Carolina Public Health Leadership Institute*. UNC Gillings School of Global Public Health. Retrieved July 25, 2024, from https://sph.unc.edu/nciph/ncphli/

Uno, H., & Zakariasen, K. (2010). Public health leadership education in North America. *Journal of Healthcare Leadership,* 11. https://doi.org/10.2147/jhl.s9727

USF Health College of Public Health. (n.d.). *Center for Leadership in Public Health Practice.* Health.usf.edu. Retrieved July 25, 2024, from https://health.usf.edu/publichealth/research/clphp/overview

Wesp, L., Scheer, V., Ruiz, A., Walker, K., & Weitzel, J. (2018). *An Emancipatory Approach to Cultural Competency: The An Emancipatory Approach to Cultural Competency: The Application of Critical Race, Postcolonial, and Intersectionality Application of Critical Race, Postcolonial, and Intersectionality Theories Theories*. https://epublications.marquette.edu/cgi/viewcontent.cgi?article=1707&context=nursing_fac#:~:text=1%2C19%20An%20emancipatory%20approach

Western Governors University (WGU). (2021, January 15). *What Is Adaptive Leadership?* Western Governors University. https://www.wgu.edu/blog/what-adaptive-leadership2101.html

Whitney, D. K., Trosten-Bloom, A., & Rader, K. (2010). *Appreciative leadership : focus on what works to drive winning performance and build a thriving organization*. Mcgraw-Hill.

Wright, K., Rowitz, L., Merkle, A., Reid, W. M., Robinson, G., Herzog, B., Weber, D., Carmichael, D., Balderson, T. R., & Baker, E. (2000). Competency development in public health leadership. *American Journal of Public Health,* 90(8), 1202–1207. https://www.ncbi.nlm.nih.gov/pmc/articles/PMC1446322/